

Nae: [NAME]
Study! Date: [DATE]
ment 1D: 206116

[LOCATION] [NAME][DATE] -

MRIS MRA EXAMINATION OE!

- * Sagittal T2WI
- * Axial T2 and FLAIR WI
- * Coronal TW

* TOF MRA

FINDINGS:

LM

Diffusion restriction at the left hemipons measuring 14 x 11 mm being very bright in [HOSPITAL] and mild altered signal in rest of enclosed sequences)

Large retro-cerebellar cystic lesion prominent [HOSPITAL] likely measuring 7x 2.cm. Multiple signal voids scattered at the cerebral parenchyma in [LOCATION].

+ Periventricular thin sheet of high 12 and [HOSPITAL] signal are seen [HOSPITAL] frontal and parietal small periventricular and subcortical foci

+ Bi

and [HOSPITAL] signal are seen neithet surrounded by edema nor exerting mass effec me of the foci show CSF like signal.

* Prominent cerebral ventricles and mildly dilated cortical sulci and extra axial [HOSPITAL] spaces.

* Normal size and parenchymal signal intensity pattern of the cerebellum, and brain stem,

- * No shift of the midline structures,
- * No intra or extra axial <NRP>.0,15.
- * No extra-axial collections.

* intact cranio-cervical junction

OPINION:

= Recent left hemi-pontine infracton

Large retroverbelay rather arachnoid cyst

* Picture also suggestive of amyloid an

jopathy for clinical correlation

© Small artery disease with bilateral cerebral ischemic foci and old lacunar infarcts,

* Brain involutinal changes,

Much Obliged

Dr.[NAME],[LOCATION],